PRIOR AUTHORIZATION REQUEST

Cover Letter

Patient Name	John Smith
Date of Birth	1968-03-15
Insurance	United Healthcare
Member ID	UHC-887612
Policy Number	UHC-POL-2026-001
CPT Code	75563
ICD-10	I42.0
Physician	Dr. Ramesh Kumar
Physician NPI	1234567890
Facility	City Heart Institute
Date of Request	March 25, 2026

March 25, 2026

United Healthcare Prior Authorization Department

Radiology Prior Authorization Team

SUBJECT: Prior Authorization Request for Cardiac MRI with Contrast (CPT 75563)

Patient: John Smith, DOB: 1968-03-15, Member ID: UHC-887612

Dear Prior Authorization Review Team,

I am writing on behalf of my patient, Mr. John Smith, to request prior authorization for a Cardiac MRI with Contrast (CPT 75563).

Patient Information:

Patient Name: John Smith

Date of Birth: 1968-03-15

Insurance Payer: United Healthcare

Member ID: UHC-887612

Policy Number: UHC-POL-2026-001

Group Number: GRP-7721

Plan Name: UHC Gold Plus

Requesting Physician Information:

Physician Name: Dr. Ramesh Kumar

NPI: 1234567890

Specialty: Cardiology

Facility: City Heart Institute

Procedure Requested:

CPT Code: 75563

Procedure Name: Cardiac MRI with Contrast

Primary Diagnosis: Dilated Cardiomyopathy (ICD-10: I42.0)

Mr. John Smith is a 58-year-old male with a documented 3-year history of Dilated Cardiomyopathy (ICD-10: I42.0). Despite strict adherence to optimal Guideline-Directed Medical Therapy (GDMT) for over three years, including Carvedilol 25mg BID, Lisinopril 10mg daily, Spironolactone 25mg daily, and Furosemide 40mg daily, Mr. Smith continues to exhibit persistent symptoms consistent with New York Heart Association (NYHA) Class III heart failure. A recent transthoracic echocardiogram performed on January 15, 2026, documented a severely reduced Left Ventricular Ejection Fraction (LVEF) of 35% with diffuse global hypokinesis and moderately dilated left ventricle. Recent lab results from March 10, 2026, show a significantly elevated BNP level of 450 pg/mL, further supporting ongoing moderate to severe heart failure. While the echocardiogram provided a baseline assessment, it is insufficient to fully characterize myocardial tissue, identify specific fibrosis patterns, or definitively rule out infiltrative cardiomyopathies. Therefore, a Cardiac MRI with contrast (CPT 75563) is medically necessary to provide high-resolution tissue characterization, evaluate myocardial fibrosis via Late Gadolinium Enhancement (LGE), assess myocardial viability for potential revascularization candidacy, and differentiate between ischemic and non-ischemic cardiomyopathy. The critical information obtained from this advanced imaging study will directly impact the clinical treatment plan, influence prognosis, and guide future high-stakes interventional therapies, including potential consideration for ICD implantation.

Please find attached the following supporting documentation for your review:

* Clinical Justification Note from Dr. Ramesh Kumar, dated March 16, 2026

* Transthoracic Echocardiogram Report, dated January 15, 2026

* Laboratory Results Report, dated March 10, 2026

* Physician Order Letter, dated March 16, 2026

* Prior Treatment Records – Cardiac Medication History, dated March 16, 2026

As per your policy guidelines, all required documentation has been submitted at the time of this request to avoid administrative delays. Submissions can be made via Online Portal (UHCprovider.com/priorauth), Phone (1-866-889-8054), Fax (1-855-820-4189), or EDI Transaction Set 278.

Thank you for your prompt attention to this matter. Please do not hesitate to contact my office at 555-123-4567 if you require any further information.

Sincerely,

Dr. Ramesh Kumar, MD

NPI: 1234567890

Board Certified Cardiologist

City Heart Institute

123 Medical Center Drive

Houston, Texas 77001

Clinical Summary

Clinical Summary for Prior Authorization

Patient: John Smith, DOB: 1968-03-15 (58 y/o Male)

Payer: United Healthcare, Member ID: UHC-887612

Requesting Physician: Dr. Ramesh Kumar, Cardiology, NPI: 1234567890

Facility: City Heart Institute

Procedure Requested: Cardiac MRI with Contrast (CPT 75563)

1. Diagnosis:

* I42.0: Dilated Cardiomyopathy

* I50.20: Heart Failure with Reduced Ejection Fraction

2. Patient History:

Mr. Smith is a 58-year-old male with a 3-year history of Dilated Cardiomyopathy, diagnosed in January 2023. He presents with chronic and progressive symptoms consistent with New York Heart Association (NYHA) Class III heart failure, including worsening dyspnea on exertion, persistent fatigue, reduced exercise tolerance, orthopnea requiring 3-pillow support, and bilateral lower extremity edema. He has been strictly compliant with Guideline-Directed Medical Therapy (GDMT) for over three years, including Carvedilol 25mg BID, Lisinopril 10mg QD, Spironolactone 25mg QD, and Furosemide 40mg QD. Attempts to escalate therapy with Sacubitril/Valsartan and Ivabradine were unsuccessful due to symptomatic hypotension/dizziness or inadequate heart rate reduction, respectively.

3. Reason for Procedure (CPT 75563):

Cardiac MRI with contrast (CPT 75563) is requested to provide definitive myocardial tissue characterization, assess for myocardial fibrosis via Late Gadolinium Enhancement (LGE), evaluate myocardial viability, and differentiate between ischemic and non-ischemic cardiomyopathy. This advanced imaging is crucial to guide future high-stakes treatment decisions, including potential revascularization or implantable cardioverter-defibrillator (ICD) placement, given the patient's persistent symptoms despite optimal GDMT.

4. Supporting Evidence:

* **Echocardiogram (Jan 15, 2026):** Documented severely reduced Left Ventricular Ejection Fraction (LVEF) of 35% with diffuse global hypokinesis and a moderately dilated left ventricle (LV End Diastolic Diameter: 6.2 cm). The report concluded that echocardiography is "insufficient to fully characterize the myocardial tissue" and recommended Cardiac MRI for further tissue characterization.

* **Lab Results (March 10, 2026):** Significantly elevated BNP at 450 pg/mL (Reference: <100 pg/mL) and NT-proBNP at 1850 pg/mL (Reference: <125 pg/mL for age <75), consistent with moderate to severe heart failure.

* **Clinical Course:** Persistent NYHA Class III symptoms and reduced functional capacity despite 3 years of maximal tolerated GDMT.

5. Clinical Justification:

This Cardiac MRI with contrast is medically necessary and aligns with standard of care guidelines for patients with Dilated Cardiomyopathy and persistent heart failure symptoms despite optimal GDMT, where echocardiography is insufficient for comprehensive tissue characterization. The detailed information on myocardial fibrosis, viability, and etiology provided by CMR is essential for accurate prognosis, risk stratification, and guiding advanced therapeutic interventions, thereby directly impacting the patient's management and outcomes.

Requirements Checklist

Requirement	Status	Evidence
Clinical Justification Note: Detailed note from treating Cardiologist explaining medical necessity, including patient history, current symptoms, previous treatments tried, reason echocardiogram is insufficient. Must be signed and dated.	✔ MET	Clinical Justification Note from Dr. Ramesh Kumar MD (Cardiologist) dated March 16, 2026, is signed and details 3-year history of Dilated Cardiomyopathy (DCM), current NYHA Class III symptoms (shortness of breath on exertion, fatigue, reduced exercise tolerance, palpitations), previous treatments (Carvedilol, Lisinopril, Spironolactone, Furosemide for 3+ years with inadequate response), and explains echocardiogram insufficiency (LVEF 35%, global hypokinesis, insufficient for tissue characterization, fibrosis, viability).
Echocardiogram Report: Most recent echocardiogram report within last 12 months. Must document LVEF value, chamber dimensions, wall motion analysis, and valve function.	✔ MET	Echocardiogram Report dated January 15, 2026 (within 12 months) documents LVEF of 35%, LV End Diastolic Diameter 6.2 cm, LV End Systolic Diameter 5.1 cm, diffuse global hypokinesis, mild mitral/tricuspid regurgitation, and normal aortic/pulmonic valve function.
Recent Lab Results: Laboratory results within last 30 days including: BNP or NT-proBNP levels, complete metabolic panel, complete blood count.	✔ MET	Laboratory Results Report dated March 10, 2026 (within 30 days) includes BNP 450 pg/mL, NT-proBNP 1850 pg/mL, a complete metabolic panel (normal creatinine 1.1 mg/dL, eGFR 72 mL/min, normal electrolytes), and a complete blood count (all normal).
Physician Order Letter: Official order on physician letterhead signed by referring Cardiologist. Must include: procedure requested, ICD-10 diagnosis code, clinical indication, physician NPI number.	✔ MET	Physician Order Letter on City Heart Institute letterhead from Dr. Ramesh Kumar MD (Board Certified Cardiologist, NPI: 1234567890) dated March 16, 2026, requests Cardiac MRI with Contrast (CPT 75563) for Dilated Cardiomyopathy (ICD-10: I42.0) with clinical indication for tissue characterization, fibrosis, and viability assessment.
Prior Treatment Records: Documentation of cardiac medications tried for minimum 3 months including drug names, dosages, duration of therapy, and patient response or reason for discontinuation.	✔ MET	Prior Treatment Records dated March 16, 2026, document Carvedilol, Lisinopril, Spironolactone, and Furosemide tried for 3+ years with dosages, duration, and partial response/persistence of symptoms. It also lists Sacubitril/Valsartan and Ivabradine with reasons for discontinuation.
Eligibility Criteria: Patient must have a confirmed diagnosis of one of the following conditions: Dilated Cardiomyopathy (ICD-10: I42.0), Hypertrophic Cardiomyopathy (ICD-10: I42.1), Restrictive Cardiomyopathy (ICD-10: I42.5), Cardiac Sarcoidosis (ICD-10: D86.85), or Suspected Cardiac Tumor or Mass.	✔ MET	Patient has a confirmed diagnosis of Dilated Cardiomyopathy (ICD-10: I42.0) as stated in the EHR data, Clinical Justification Note, and Physician Order Letter.

Eligibility Criteria: Symptoms (Shortness of breath on exertion, Chest pain or pressure, Palpitations or arrhythmia, Unexplained syncope or near-syncope, Reduced exercise tolerance) must be present and documented for a minimum period of 3 months.	✔ MET	Clinical Justification Note documents a 3-year history of Dilated Cardiomyopathy with worsening shortness of breath on exertion, persistent fatigue, reduced exercise tolerance, and occasional palpitations, consistent with NYHA Class III symptoms.
Eligibility Criteria: Patient must have completed an echocardiogram prior to the MRI request.	✔ MET	An echocardiogram was completed on January 15, 2026, which is prior to the MRI request date of March 16, 2026.
Eligibility Criteria: The echocardiogram must show a documented LVEF (Left Ventricular Ejection Fraction) value and evidence of wall motion abnormality or structural cardiac abnormality requiring further characterization.	✔ MET	Echocardiogram report dated January 15, 2026, documents an LVEF of 35% and diffuse global hypokinesis, along with a moderately dilated left ventricle.
Eligibility Criteria: The request must be ordered by a board-certified Cardiologist or Cardiac Electrophysiologist.	✔ MET	The request is ordered by Dr. Ramesh Kumar MD, who is a Board Certified Cardiologist, as stated in the EHR and supporting documents.
Eligibility Criteria: Cardiac MRI is covered when echocardiogram findings are inconclusive.	✔ MET	The Clinical Justification Note states the echocardiogram is 'insufficient to fully characterize the myocardial tissue or identify specific fibrosis patterns' and 'lacks the resolution required to assess myocardial viability'. The Echocardiogram Report also recommends MRI for 'further tissue characterization'.
Eligibility Criteria: Cardiac MRI is covered for assessment of myocardial viability.	✔ MET	The Clinical Justification Note and Physician Order Letter explicitly state the MRI is for 'assessment of myocardial viability' and 'critical to assess myocardial viability for potential revascularization candidacy'.
Eligibility Criteria: Cardiac MRI is covered for evaluation of cardiac masses or tumors.	✘ MISSING	The patient's clinical justification for the MRI does not explicitly state evaluation of cardiac masses or tumors as the primary indication. The justification focuses on myocardial fibrosis, viability, and differentiating ischemic/non-ischemic cardiomyopathy.
Eligibility Criteria: Cardiac MRI is covered for assessment of pericardial disease.	✘ MISSING	While a small pericardial effusion is noted on the echocardiogram, the primary clinical indication for the MRI, as stated in the justification and order, is not for the assessment of pericardial disease.
Eligibility Criteria: Cardiac MRI is covered for congenital heart disease evaluation.	✘ MISSING	The patient's diagnosis is Dilated Cardiomyopathy (ICD-10: I42.0), not congenital heart disease.
Eligibility Criteria: Repeat MRI within 6 months is not covered without new symptoms.	✔ MET	No prior Cardiac MRI within the last 6 months is documented in the provided EHR data, indicating this is an initial request.

Attached Documents

The following documents are attached to this package:

#		
1	Clinical Justification Note Kumar	Clinical_Justification_Note_Kumar.pdf
2	Echocardiogram Report Jan2026	Echocardiogram_Report_Jan2026.pdf
3	Laboratory Results Report	Laboratory_Results_Report.pdf
4	Physician Order Prior Auth Cardiac Mri	Physician_Order_Prior_Auth_Cardiac_MRI.pdf
5	Prior Treatment Records Cardiac Medication	Prior_Treatment_Records_Cardiac_Medication.pdf

City Heart Institute

123 Medical Center Drive
Phone: 555-123-4567
Department: Cardiology

Physician: Dr. Ramesh Kumar MD
NPI Number: 1234567890
Date: March 16, 2026

RE: PRIOR AUTHORIZATION REQUEST FOR CARDIAC MRI WITH CONTRAST (CPT 75563)

PATIENT HISTORY

The patient is a 58-year-old male with a documented 3-year history of Dilated Cardiomyopathy (ICD-10: I42.0). Initially diagnosed in January 2023 following a presentation with acute dyspnea, the patient has since exhibited a chronic and progressive clinical course. Current presentation is significant for worsening shortness of breath on exertion, persistent fatigue, and a marked reduction in exercise tolerance that interferes with activities of daily living. Furthermore, the patient reports occasional palpitations, necessitating advanced diagnostic evaluation to assess for structural triggers or arrhythmogenic substrates.

CURRENT SYMPTOMS

At present, the patient exhibits symptoms consistent with New York Heart Association (NYHA) Class III heart failure. Clinical findings include shortness of breath both at rest and upon minimal physical exertion, as well as significant orthopnea requiring 3-pillow support for sleep. Physical examination reveals bilateral lower extremity edema, indicating volume overload. Recent diagnostic monitoring confirms a persistently reduced ejection fraction, signifying a lack of improvement despite standard therapeutic interventions.

PREVIOUS TREATMENTS TRIED

The patient has remained strictly compliant with Guideline-Directed Medical Therapy (GDMT) since diagnosis. The therapeutic regimen includes the following:

- Carvedilol: 25mg twice daily (initiated January 2023)
- Lisinopril: 10mg daily (initiated January 2023)
- Spironolactone: 25mg daily (initiated March 2023)
- Furosemide: 40mg daily for fluid management

Despite adherence to optimal medical therapy for over three years, the patient's symptoms persist and have shown recent signs of progression, suggesting the underlying pathology requires more granular characterization than what is provided by current management tools.

CLINICAL INSUFFICIENCY OF ECHOCARDIOGRAPHY

A transthoracic echocardiogram (TTE) performed in January 2026 documented a Left Ventricular Ejection Fraction (LVEF) of 35% with global hypokinesis. While the TTE provided a baseline assessment of chamber size and function, it is insufficient to fully characterize the myocardial tissue or identify specific fibrosis patterns. Standard echocardiography lacks the resolution required to assess myocardial viability or definitively rule out infiltrative cardiomyopathies. Consequently, a Cardiac MRI with contrast is essential for definitive tissue characterization and to provide the high-resolution data necessary to guide future high-stakes treatment decisions.

MEDICAL NECESSITY STATEMENT

Cardiac MRI with contrast is medically necessary for this patient to evaluate myocardial fibrosis via Late Gadolinium Enhancement (LGE). This study is critical to assess myocardial viability for potential revascularization candidacy and to differentiate between ischemic and non-ischemic cardiomyopathy. The findings from this advanced imaging will directly impact the clinical treatment plan, influence prognosis, and determine the appropriateness of further interventional therapies.

Respectfully,

Dr. Ramesh Kumar MD
Date Signed: March 16, 2026
Credentials: Board Certified Cardiologist
License Number: MD-12345-TX

TRANSTHORACIC ECHOCARDIOGRAM REPORT

Ordering Physician:	Dr. Ramesh Kumar MD	**Study Date:**	January 15, 2026
Performing Physician:	Dr. Sarah Johnson MD	**Report Date:**	January 15, 2026
Facility:	City Heart Institute	**Study Type:**	Complete TTE

Clinical Indication: Dilated Cardiomyopathy; Follow up evaluation

SECTION 1: TECHNICAL QUALITY

Study Quality: Adequate
Windows: Acceptable acoustic windows obtained.
Views: Parasternal long axis, short axis, apical 4-chamber, 2-chamber, 3-chamber, and subcostal views were successfully obtained and analyzed.

SECTION 2: LEFT VENTRICLE MEASUREMENTS

Measurement	Result	Normal Reference Range
LV End Diastolic Diameter	6.2 cm	Less than 5.5
LV End Systolic Diameter	5.1 cm	Less than 3.9
Interventricular Septum	0.8 cm	0.6 to 1.1
Posterior Wall Thickness	0.8 cm	0.6 to 1.1
LV Ejection Fraction (LVEF)	**35%**	Greater than 55%
LV Fractional Shortening	18%	25 to 45%

SECTION 3: LEFT VENTRICLE FUNCTION

LVEF: 35% — Severely reduced.
Wall Motion: Diffuse global hypokinesis noted. No regional wall motion abnormalities (RWMA) identified.
Diastolic Function: Grade II diastolic dysfunction (pseudonormal pattern).
LV Size: Moderately dilated left ventricle.

SECTION 4: RIGHT VENTRICLE

RV Size: Normal.
RV Function: Mildly reduced.
TAPSE: 1.6 cm (mildly reduced).
RV Systolic Pressure: Estimated 38 mmHg.

SECTION 5: VALVES

Mitral Valve: Mild mitral regurgitation identified. Etiology appears functional, secondary to LV dilation and annular stretching.
Aortic Valve: Trileaflet, normal opening. No significant stenosis or regurgitation visualized.
Tricuspid Valve: Mild tricuspid regurgitation.
Pulmonic Valve: Normal appearance and function.

SECTION 6: OTHER FINDINGS

Pericardium: Small pericardial effusion noted.
Aorta: Normal aortic root diameter.
IVC: Dilated IVC suggesting elevated right atrial pressure.

SECTION 7: CONCLUSION

1. **Severely reduced left ventricular systolic function with LVEF of 35%.**
2. **Moderately dilated left ventricle consistent with dilated cardiomyopathy.**
3. **Diffuse global hypokinesis.**
4. **Mild mitral regurgitation — functional etiology.**
5. **Grade II diastolic dysfunction.**
6. **Small pericardial effusion.**
7. **Recommend Cardiac MRI for further tissue characterization.**

Interpreting Physician: Dr. Sarah Johnson MD
Board Certified Echocardiographer
Date: January 15, 2026

CITY HEART INSTITUTE

Clinical Pathology Laboratory

Laboratory Results Report

Clinical Laboratory Report

Ordering Physician:	Dr. Ramesh Kumar MD	**Specimen Type:**	Venous Blood
Collection Date:	March 10, 2026	**Clinical Indication:**	Dilated Cardiomyopathy
Report Date:	March 10, 2026		Cardiac MRI Pre-authorization

SECTION 1 - CARDIAC BIOMARKERS

Test	Result	Unit	Reference Range	Flag
BNP (B-type Natriuretic Peptide)	**450**	pg/mL	Less than 100	**HIGH**
NT-proBNP	**1850**	pg/mL	Less than 125 (age < 75)	**HIGH**
Troponin I (High Sensitivity)	0.04	ng/mL	Less than 0.04	**BORDERLINE**

SECTION 2 - COMPLETE BLOOD COUNT

Test	Result	Unit	Reference Range	Flag
WBC	7.2	10^3/uL	4.5 to 11.0	NORMAL
RBC	4.8	10^6/uL	4.5 to 5.9	NORMAL
Hemoglobin	13.8	g/dL	13.5 to 17.5	NORMAL
Hematocrit	41.2	%	41 to 53	NORMAL
Platelets	215	10^3/uL	150 to 400	NORMAL
MCV	88	fL	80 to 100	NORMAL

SECTION 3 - COMPREHENSIVE METABOLIC PANEL

Test	Result	Unit	Reference Range	Flag
Sodium	138	mEq/L	136 to 145	NORMAL
Potassium	4.1	mEq/L	3.5 to 5.1	NORMAL
Creatinine	1.1	mg/dL	0.7 to 1.3	NORMAL
eGFR	72	mL/min	Greater than 60	NORMAL
BUN	18	mg/dL	7 to 20	NORMAL

Test	Result	Unit	Reference Range	Flag
Glucose	95	mg/dL	70 to 100	NORMAL
AST	28	U/L	10 to 40	NORMAL
ALT	22	U/L	7 to 56	NORMAL
Albumin	3.8	g/dL	3.5 to 5.0	NORMAL

SECTION 4 - LIPID PANEL

Test	Result	Unit	Reference Range	Flag
Total Cholesterol	195	mg/dL	Less than 200	NORMAL
LDL	118	mg/dL	Less than 130	NORMAL
HDL	42	mg/dL	Greater than 40	NORMAL
Triglycerides	**175**	mg/dL	Less than 150	**BORDERLINE HIGH**

SECTION 5 - INTERPRETATION NOTE

Significantly elevated BNP at 450 pg/mL and NT-proBNP at 1850 pg/mL consistent with moderate to severe heart failure.
Renal function preserved. Electrolytes within normal limits.
Results support ongoing heart failure management and prior authorization for advanced cardiac imaging.

Authorized By:

Dr. Michael Chen MD
Board Certified Pathologist
Date: March 10, 2026

CITY HEART INSTITUTE

Department of Cardiology | 123 Medical Center Drive | Houston, Texas 77001
Phone: 555-123-4567 | Fax: 555-123-4568 | www.cityheartinstitute.com

Date: March 16, 2026

To:
United Healthcare Prior Authorization Department
Radiology Prior Authorization Team

From:
Dr. Ramesh Kumar MD
Board Certified Cardiologist
NPI: 1234567890 | License: MD-12345-TX | DEA: RK1234563

SUBJECT: PRIOR AUTHORIZATION REQUEST
PROCEDURE: CARDIAC MRI WITH CONTRAST (CPT 75563)
ICD-10 CODE: I42.0

REQUESTED PROCEDURE

Name:	Cardiac MRI with Contrast
CPT Code:	75563
Urgency:	Routine — Non Emergency
Facility:	City Heart Institute Radiology
Estimated Date:	Within 30 days of approval

CLINICAL INDICATION

Primary Diagnosis: Dilated Cardiomyopathy (ICD-10: I42.0)
Secondary Diagnosis: Heart Failure with Reduced Ejection Fraction (ICD-10: I50.20)

MEDICAL NECESSITY STATEMENT

This procedure is medically necessary for:

- Tissue characterization of myocardium
- Assessment of myocardial fibrosis via late gadolinium enhancement imaging
- Differentiation of ischemic versus non-ischemic cardiomyopathy
- Assessment of myocardial viability
- Guidance of treatment decisions including consideration of ICD implantation

SUPPORTING CLINICAL INFORMATION

- LVEF 35% on recent echocardiogram
- Elevated BNP 450 pg/mL
- NYHA Class III symptoms
- Optimal medical therapy ongoing
- No contraindications to MRI or gadolinium contrast media

> I certify that this procedure is medically necessary and appropriate for this patient. All information provided is accurate and complete to the best of my knowledge.

Dr. Ramesh Kumar MD
Board Certified Cardiologist
Date: March 16, 2026
NPI: 1234567890
Contact: 555-123-4567

CITY HEART INSTITUTE

Cardiology Department

Prior Treatment Records

Cardiac Medication History

Treating Physician:	Dr. Ramesh Kumar MD	**Report Date:**	March 16, 2026
Facility:	City Heart Institute	**Specialty:**	Cardiology
Period Covered:	Jan 2023 – Mar 2026	**Clinical Indication:**	Dilated Cardiomyopathy

This document summarizes the complete cardiac medication history for a patient with confirmed Dilated Cardiomyopathy (ICD-10: I42.0) over a period of 3 years from January 2023 to March 2026. All medications listed below were prescribed and monitored by the treating cardiologist as part of optimal medical therapy for heart failure with reduced ejection fraction.

SECTION 1 - MEDICATION HISTORY TABLE

Medication	Dose/ Freq	Dates	Duration	Response	Status
Carvedilol (Beta Blocker)	25mg Twice daily	Jan 2023 – Present	3 years	Partial response — HR controlled but symptoms persist	**CURRENT**
Lisinopril (ACE Inhibitor)	10mg Once daily	Jan 2023 – Present	3 years	Partial response — BP controlled but LVEF unchanged	**CURRENT**
Spironolactone (Aldo. Antagonist)	25mg Once daily	Mar 2023 – Present	2y 9mo	Minimal improvement in symptoms	**CURRENT**
Furosemide (Loop Diuretic)	40mg Once daily	Mar 2023 – Present	2y 9mo	Adequate fluid control; edema managed	**CURRENT**
Sacubitril/ Valsartan (ARNI)	24/26mg Twice daily	Sep 2023 – Nov 2023	2 months	Discontinued due to symptomatic hypotension and dizziness	**DISCONTINUED**
Ivabradine	5mg Twice daily	Jan 2024 – Mar 2024	2 months	Discontinued — inadequate heart rate reduction	**DISCONTINUED**

SECTION 2 - TREATMENT RESPONSE SUMMARY

Despite 3 years of optimal medical therapy with maximum tolerated doses of guideline directed medical therapy (GDMT) including beta blocker, ACE inhibitor, and aldosterone antagonist, patient continues to exhibit:

- Persistent LVEF of 35%
- NYHA Class III symptoms (marked limitation in activity)
- Elevated BNP levels (refer to lab report dated 03/10/2026)
- Reduced functional capacity

SECTION 3 - CURRENT MEDICATION LIST

- **Carvedilol:** 25mg twice daily
- **Lisinopril:** 10mg once daily
- **Spironolactone:** 25mg once daily
- **Furosemide:** 40mg once daily
- **Aspirin:** 81mg once daily
- **Atorvastatin:** 40mg once daily

Physician Attestation:
I certify that the above medication history is accurate and complete. The patient has received optimal guideline directed medical therapy for a minimum of 3 years with inadequate clinical response. Advanced cardiac imaging (Cardiac MRI) is required to guide further treatment decisions, including viability assessment and potential ICD placement.

Dr. Ramesh Kumar MD
Board Certified Cardiologist | NPI: 1234567890
Date: March 16, 2026